Name: **Sex:** **Race:** **Address:**

DOB: **TOB:** **CGA:**

Maternal details: Gestational status: GPLA: LMP: EDD:

Mother's age: Father's age:

Blood group of mother and father:

Maternal illness or drug intake

Antenatal history:

1st trimester: excessive vomiting, fever with rash (may indicate intra uterine infection), drug intake, radiation exposure, bleeding per vaginum, increased frequency of micturition

2nd trimester: history suggestive of PIH, GDM, quickening, USG abdomen, anemia in pregnancy

3rd trimester: immunization (TT), bleeding or leaking PV, increased frequency of micturition, leaking PV with fever or pain in lower abdomen

Weight gain in pregnancy

Natal history:

Duration of labor, Spontaneous/induced, Vertex/breech, Liqor colour

Term or preterm, Mode of delivery – NVD, instrumental, operative, place of delivery – home / hospital & who conducted, cried immediately at birth or not, resuscitative measures required, birth weight, time of initiation of breast feeding

Neonatal history: Early neonatal & late neonatal history:

Any neonatal complications, neonatal hyper-bilirubinemia / seizures /cyanosis/ any other complications & treatment given, whether child was mother side or under NICU care, day of life when child was discharged, feeding and activity of the child, first urine and meconium passed on _____ day of life.

If any complications are there, detailed history including negative history should be mentioned

Activity and feeding pattern should be mentioned.

Treatment given to the baby in NICU/duration of hospital stay

Diet or feeding history:

Whether exclusively breast or supplemental feeds/any pre-lacteals given

No. of times the mother feeds the child and urine output of the child.

How long after each feed, child sleeps?

Whether there is any regurgitation or possetting?

Family history:

Consanguinity/family h/o splenectomy or any surgery in family members

Pedigree chart

Immunization history:

BCG given or not, OPV and Hepatitis B vaccine given or not

Socio-economic history: as per modified Kuppuswamy classification

Examination:

General statement including state of wakefulness (Brazelton)

Vitals – HR, RR, Temperature and CRT

Anthropometry:

Weight, length, HC, CC

General examination:

Colour, Pallor/icterus/cyanosis/edema

Gestational assessment: Modified Ballard scoring (<72 hours)

Head to toe examination:

Skull – size, shape, sutures, AF, PF, any other swellings (Caput/cephahematoma)

Eyes – discharge, any abnormalities, red reflex (using ophthalmoscope)

Ears – low set or abnormal shaped

Nose – bilateral choanae patency

Oral cavity – cleft lip/palate/natal teeth/oral thrush

Neck – swelling or webbing

Skin – icterus (Krammer's rule), milia, Mongolian spots, hemangioma, pustules

Umbilicus – shriveled/foul smell/peri-umbilical erythema/bleeding/discharge/umbilical hernia

Genitalia – descended or undescended, urethra

Hips – DDH (Ortolani/Barlow)

Femoral pulses – bilateral

Arms and legs – shortening of any fingers/Dermatoglyphics

Bones and spine – neural tube defects/tuft of hair/pits

Systemic examination

CNS examination:

Neurological assessment: (normal resting posture, abnormal posture - abduction of thighs (frog legs), arms lie in flaccid position besides the head

State of wakefulness:

I – eyes closed, regular respirations, no mvts

II – eyes closed, irregular respiration, no gross mvt

III – eyes open, no gross mvt

IV – eyes open, gross mvts, no crying

V – Eyes open or closed, crying

CRANIAL NERVES

II, III, IV, VI – blink response to light (abnormal – squint, no blink response, gross nystagmus)

V – Normal corneal reflex

VII – deviation of angle of mouth while crying

VIII – startle response to loud noise

X, XII – ability to suck, swallow and normal rooting reflex

MOTOR SYSTEM

Tone – resistance of muscle to stretch – resistance to extension is normal with recoil to flexed position (abnormal – absence of recoil and increased resistance)

Phasic tone – tendon reflexes – absence indicates either dysfunction of motor unit or cerebral disorders as in HIE

Ankle clonus – if sustained indicates early spasticity

Postural tone – low amplitude sustained stretch reflex – normal – traction response

Vertical suspension

Horizontal suspension

REFLEXES

Moro's

STNR

Crossed extensor

Peripheral nerves

Moro's – unilateral indicates Erb's, Klumpke's paralysis

180 degree flip examination